



INPATIENT RECORD

Hospital No: 57233772	Visit No:
Name:	Age/Sex:
Doctor Name:	Specialty:
Date:	

Pallor: 02	Cyanosis: 02	Jaundice, Neonatal (Appearance): 02
Erythema: 02	Petechiae: 02	Oedema: 02
Surgical scars: 02	Injury treatment: 02	Burns: 02
Rash: 02		

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 10
Remarks : Moderate Fall Risk
Age : 70-79 Years - 2

Medications:

ON 1 HIGH FALL RISK DRUG - 3
Patient care Equipment : Two Present - 2
Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2
Cognition : Altered awareness of immediate physical environment- 1
Type of Assessment : Initial Assessment
Pain Assessments
Score: 1
Type of Assessment: Initial Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 1

Additional Pain Assessment Details

Time: With position changes

Skin Examination

Pallor : No
Cyanosis : No



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Visit No:
Doctor Name:

Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : No
Burns : No
Other : REDNESS ON BOTH HANDS
Hair : Normal

Nails : Normal

NURSING INITIAL ASSESSMENT - NEW

Status : Entered
Score : 0
Patient Name : DR SHIVARAJ K S
MRN : MH009742499
Age : 74 Yrs
Gender : Male
Patient arrival Date : 24/03/2026
Patient Arrival Time : 07:15
Accompanied by Relative : Yes
Name of Relative : MR ROSHAN K S
Relationship with Patient : SON
Contact Number : 9620550446
Primary Language Spoken : URDU
Interpreter Needed : No
Room Orientation Given : Yes
Patient Identification Band Fixed : Yes
If Yes Type Of Band : GREEN ID BAND
Height(Cm) : ..
Weight(Kgs) : ..
Reason Not Able To Check : PATIENT IS ON OXYGEN
Chief Complaints : BREATHING DIFFICULTY
Past History : UPPER GI BLEED - S/P OGD SHOWED GE JUNCTION BLEEDING LESION NOTED -
HEMOCLIP APPLIED [20/11/25]
HYPERTENSION
PREDIABETIC
Vital Assessment
Temp(F) : 98.3
Pulse/min : 105
Blood Pressure(mmHg) : 140/90
Respiration/min : 23
Spo2(%) : 96



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Visit No:

Name:

Doctor Name:

GRBS (mg/dl) : 117
 Skin Integrity : INTACT
 Other Parameters
 EWS Score : 5
 GCS : E4M6V5
 Pain Score : 1
 Valuable handover checklist completed : Yes
 Belongings : No
 Dentures : No
 Hearing Aid : No
 Eye Glasses/Contact Lens : No
 Others : ..
 Vulnerable : Yes
 Any Special Care Given : AGED
 Restraints : No
 If Yes, Type Of Restraint : N/A
 Any Known Allergies : UNKNOWN
 Current Medications : TAB FLUVIR 75 MG P/O 1-0-1
 Informed the Doctor : Yes
 Name of the Doctor : Dr. Ashley Elizabeth Thomas
 Investigations Ordered : ECG, CBC, RP2, ABG, URINE ROUTINE
 RADIOLOGY INVESTIGATIONS: CXR
 Diet : SOFT DIET
 Collected Old records : Yes
 If Yes Document Type : OP FILE
 Is the patient at risk for any of the following? If Yes, complete the risk assessment form.
 Fall : YES
 Pressure Score : YES
 DVT : YES
 Aspiration : YES
 Pain Assessment Score : Acute
 Ability to perform activities of daily living
 Activity
 Bathing : Assisted
 Dressing : Assisted
 Eating : Assisted
 Walking : Assisted
 Toilet Use : Assisted
 Nursing needs(complete Nursing care plan based on the identified needs)
 Actual needs : POSITION
 AIRWAY CLEARANCE TECHNIQUES
 OXYGEN THERAPY
 MEDICATION ADMINISTRATION

 Potential needs : PATIENT EDUCATION
 INFECTION PREVENTIOIN
 FOLLOW UP CARE



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Visit No:

Name:

Doctor Name:

NUTRITIONAL SUPPORT

Any other needs : TO PROVIDE PSYCHOLOGICAL SUPPORT

Assessment Done By : SK Anisuddin

Rating : 1

BMI : NaN..

Fall Score : 10/MODERATE

Pressure score : 13 Moderate Risk

DVT Score : 5/HIGHEST

Aspiration score : 2/LOW

General Physical Examination:

Pallor : NO

Icterus : NO

Edema : NO

Clubbing : NO

Cyanosis : NO

Lymphadenopathy : NO

Any Other Findings : ..

Questionnaires

Status : Entered

Date : 24/03/2026

Patient's Identification No. : MH009742499

Sex : Male

A - Each Risk Factor Represents 1 Point : Sepsis (< 1 month),Medical patient currently at bed rest,Other risk factor

Total Tickmarks : 3

Total Score A : 3

B - Each Risk Factor Represents 2 Point : Age 60 - 74 years

Total Tickmarks : 1

Total Score B : 2

Total Score (A+B+C+D+E) = : 5

Risk Assessment : : Highest

5 or More Highest : Highest

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Score : 0

CATEGORIES

EATING HABILTS : Low Level of alertness that impairs complete participation in oral intake - 1,Needs assistance to eat - 1

TOTAL SCORE : 2

COMMENTS : Low Risk

Adult Pressure Ulcer Braden Scale

SENSORY PERCEPTION : 4

4 - No Impairment : Y

MOISTURE : 3

3 - Occasionally Moist : Y

ACTIVITY : 1

1 - Bed Bound : Y



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MOBILITY : 2
2 - Very Limited : Y
NUTRITION : 2
2 - Probably Inadequate : Y
FRICTION AND SHEAR : 1
1 - Problem : Y
TOTAL SCORE : 13 Moderate Risk

Forms

Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : No
Patient Measurable Goal for the day : TO PREVENT RISK OF DVT
Nursing Diagnosis : Risk for DVT
Nursing Interventions : a) Educate in proper use and function of assistive devices.
b) Provide progressive mobilization.
c) Motivate the patient to gradually increase mobility

Evaluation : Continue,Modify
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : No
Nursing Care Plan discussed with Family : No
Patient Measurable Goal for the day : TO PREVENT RISK OF PRESSURE ULCER
Nursing Diagnosis : Risk of pressure ulcer/Injury can cause by lack of mobilization due to pain and altered nutritional diet
Nursing Interventions :
1. ASSESS THE PATIENT CONDITION
2. EVERY 2ND HOURLY CHANGE PATIENT POSITION
3. KEEP SKIN DRY AND CLEAN
4. APPLY POWDER

Evaluation : Continue,Modify
Add Nursing Care Plan
Status : Entered
Patient Measurable Goal for the day : to restore normal breathing pattern
Nursing Diagnosis : Ineffective airway clearance
Nursing Interventions : a) Monitor respiratory rate, depth, effort, rhythm,
Breath sounds and cyanosis.
b) Promote optimal level of activity for best possible lung
Expansion, elevate the head of the bed by 45 degree.
c) Encourage cough and the breathing exercise.
d) Monitor oxygen if required and suction if required.

Evaluation : Continue,Modify

Forms



Hospital No: 57233772
Name:

Visit No:
Doctor Name:

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 9
Remarks : Moderate Fall Risk
Age : 70-79 Years - 2

Medications:

ON 1 HIGH FALL RISK DRUG - 3
Patient care Equipment : One Present- 1
Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2
Cognition : Altered awareness of immediate physical environment- 1
Type of Assessment : Shift Assessment
Pain Assessments
Score: 1
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 1

Additional Pain Assessment Details

Skin Examination
Pallor : No
Cyanosis : No
Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : No
Burns : No
Other : REDNESS ON BOTH HANDS
Hair : Normal



Hospital No: 57233772
Name:

Visit No:
Doctor Name:

Nails : Normal

Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :21

Questionnaires

Status : Entered

Date : 24/03/2026

Patient's Identification No. : MH009742499

Sex : Male

A - Each Risk Factor Represents 1 Point : Medical patient currently at bed rest,Other risk factor

Total Tickmarks : 2

Total Score A : 2

B - Each Risk Factor Represents 2 Point : Age 60 - 74 years

Total Tickmarks : 1

Total Score B : 2

Total Score (A+B+C+D+E) = : 4

Risk Assessment : : High

3 - 4 High : High

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Status : Entered

Score : 0

CATEGORIES

EATING HABILTS : Needs assistance to eat - 1

TOTAL SCORE : 1

COMMENTS : Low Risk

Adult Pressure Ulcer Braden Scale

Status : Entered

SENSORY PERCEPTION : 4

4 - No Impairment : Y

MOISTURE : 3

3 - Occasionally Moist : Y

ACTIVITY : 1

1 - Bed Bound : Y

MOBILITY : 2

2 - Very Limited : Y

NUTRITION : 2

2 - Probably Inadequate : Y

FRICTION AND SHEAR : 2

2 - Potential Problem : Y

TOTAL SCORE : 14



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Status : Entered

Forms

Nurses Endorsment Checklist

Status : Entered

Shift : Transfers

Situation : Elective,NON MLC,Patient Belongings Handover,Transfer Out

Comment / Chief Complaints / Diagnosis : C/O BREATHING DIFFICULTY

Background

Allergies : No

Remarks : UNKNOWN.

Diet : Yes

Remarks : normal

Previous Medications : No

Remarks : nil

Old reports added to TRAK : No

Remarks : nil

Consents : No

Remarks : nil

Cross Consultation : No

Remarks : nil

Procedures performed before transfer : Yes

Remarks : IV CANNULATION,

Vulnerable : Yes

Remarks : age74

Past Medical / Surgical History : RAD

DM

Intubation : no

Ventilation Status : ON OXYGEN

ET Size and Lip Level : no

Tracheostomy : no

ICD : no

Pig Tail : no

T Piece : no

Chest Physio : no

Treated with Inotropes : no

Blood Transfusion : no

Level of Consciousness : Oriented

GCS : E4M6V5

Vitals -TPR & BP : is stable : No

Pain Score : 1

On MET/PET List : No

Risk For FALL : Yes

Remarks : 9Moderate Fall Risk

Risk For Pressure Ulcer : Yes

Remarks : 14Moderate Fall Risk

Risk for Aspiration : Yes

Remarks : 1LOW



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Visit No:

Name:

Doctor Name:

Existence of Pressure Ulcer : No
 O2 Administration : No
 SPO2(Latest reading in the shift) : 96
 DIET : Oral
 On tube feeding : No
 IV Fluids : No
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : Yes
 Remarks : VRNFLOWN
 Self Void : No
 Foley's : No
 Drain : No
 Bowel Opened : No
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Next Capillary Blood glucose-checktime : 12:00
 Dressing Present : No
 High Risk medications : Yes
 Remarks : ANTIBIOTIC
 Is Patient on Physiotherapy : No
 Ambulation : Bed Bound
 Investigations Sent : No
 Reports Collected : No
 Care Plan : Yes
 Remarks : DVT,PRESSURE
 Pupil Reaction : 2MM
 Inotropes : No
 Ventilator : No
 Dialysis : No
 Special Needs : Yes
 Remarks : AUTO CPAP TO BE CONNECT OVERNIGHT
 Patient & Family Education : No
 Transfer : Yes
 Transfer Notes : 506A
Medications: Yes
 Remarks : ON TRETMENT SHEET
 Planned Procedure : No
 Approved follow-up : Yes
 Remarks : DR ORDERS
 Transfusion : No
 Patient belongings with health care providers : No
 Pending Reports : Yes
 Remarks : CT REPORT
 Critical Value : No
 Pharmacy / Consumables : Charged



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Discharge Plan : No

Comments : CHECK VITALS,I/O MONITOR ,GRBC CHECK ,AUTO CPAP TO BE CONNECT OVERNIGHT

Hand over by : SK Anisuddin

Handed over to : Balapdianghun

Team Lead Hand over By : Abu Haneefa B A

Team Lead Hand over To : Balapdianghun

Status : Entered

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : TO PREVENT RISK OF PRESSURE ULCER

Nursing Diagnosis : Risk of pressure ulcer/Injury can cause by lack of mobilization due to pain and altered nutritional diet

Nursing Interventions : 1. ASSESS THE PATIENT CONDITION

2. EVERY 2ND HOURLY CHANGE PATIENT POSITION

3. KEEP SKIN DRY AND CLEAN

4. APPLY POWDER

Evaluation : Continue,Modify

Add Nursing Care Plan

Status : Entered

Patient Measurable Goal for the day : to restore normal breathing pattern

Nursing Diagnosis : Ineffective airway clearance

Nursing Interventions : a) Monitor respiratory rate, depth, effort, rhythm, Breath sounds and cyanosis.

b) Promote optimal level of activity for best possible lung Expansion, elevate the head of the bed by 45 degree.

c) Encourage cough and the breathing exercise.

d) Monitor oxygen if required and suction if required.

Evaluation : Continue,Modify

Add Nursing Care Plan

Status : Entered

Patient Measurable Goal for the day : TO PREVENT RISK OF DVT

Nursing Diagnosis : Risk for DVT

Nursing Interventions : . a) Educate in proper use and function of assistive devices.

b) Provide progressive mobilization.

c) Motivate the patient to gradually increase mobility

Evaluation : Continue,Modify

IP Initial Assessment

Status : Entered

Score : 0

Patient Arrival Date : 24/03/2026

Patient Arrival Time : 07:20



Hospital No: 57233772
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Visit No:
Doctor Name:

Chief Complaints : Complaints of fever, cough non productive since 2 days
Complaints of breathlessness increased for a day.

History of Presenting illness : Dr. Shivaraj K S, aged 74 years, was apparently well till 14/3/26 when he developed exacerbation of RAD , was admitted to MHMR for 3 days, he was discharged on 17/3/26 on nebs antibiotics. He had fever with chills last night, associated with cough- scanty white sputum, breathlessness for a day, started on oxygen concentrator at home . He was brought to ER as he was tachypneic, high grade fever, spo2 room air 88- started on niv. Hence was shifted to MICU for further management.

Allergy : not knoiwn

Past Medical History : UPPER GI BLEED - S/P OGD SHOWED GE JUNCTION BLEEDING LESION NOTED - HEMOCLIP APPLIED [20/11/25]

HYPERTENSION

PREDIABETIC

Past Surgical History :-

Family History :-

Social / Personal History :-

Menstrual History :-

Previous Pregnancy :-

Current medications : nebs

Pulse /min : 80

Resp Rate/min : 24

Temp F : 99

BP mm Hg : 130/80

on O2 : 2l

GRBS mg/dL : 114

Pallor : NO

Icterus : NO

Edema : NO

Clubbing : NO

Cyanosis : NO

Lymphadenopathy : NO

Peripheral Pulse : +

Others :-

CVS : s1s2

RS : reduced air entry lower zones, no wheeze now

PA : soft

CNS : gcs 15/15

Others :-

Local Examination :-

Provisional Diagnosis : lrti- sepsis

rad infective exacerbation

Care Plan : INJ PAN 40 MG IV 1-0-0

INJ MAGNEX FORTE 1.5 GM IV 1-0-1

INJ PARA 1 GM IV 1-1-1

TAB FLUVIR 75 MG P/O 1-0-1



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Doctor Name:

NEB LEVOLIN 0.63MG INH 1-1-1-1

NEB FORACORT 0.5 MG (INH) 1-0-1(TO GARGLE THE MOUTH AFTER USE)

NEB NEBZMART G (INH) 1-0-1

TAB MONTAIR FX P/O 0-0-1

MONITOR VITALS

niv sos

Spo2 : 98

Diet Plan : oral as tolerated

Apache II scoring sheet

A.ACUTE PHYSIOLOGY SCORE : 6

B.Age : 5

C.CHRONIC HEALTH POINTS : 5

APACHEII SCORE="A(ACUTE" PHYSIOLOGY SCORE)+B(AGE POINTS)+C(CHRONIC HEALTH POINTS):16

Patient Acuity Tool

Stable patient 1 to 11

Moderate Risk Patient 12 to 22

Complex Patient 22 to 33

High Risk Patient >34

Total Score :32

Forms

Nurses Endorsment Checklist

Shift : Morning to Evening

Situation : Elective,NON MLC,Shift Handover

Comment / Chief Complaints / Diagnosis : C/O BREATHING DIFFICULTY

Background

Allergies : No

Remarks : UNKNOWN.

Diet : Yes

Remarks : normal

Previous Medications : No

Remarks : nil

Old reports added to TRAK : No

Remarks : nil

Consents : No

Remarks : nil

Cross Consultation : No

Remarks : nil

Procedures performed before transfer : Yes

Remarks : IV CANNULATION,

Vulnerable : Yes

Remarks : age74

Past Medical / Surgical History : RAD

DM

Intubation : N/A

Ventilation Status : N/A



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Visit No:

Name:

Doctor Name:

ET Size and Lip Level : N/A
 Tracheostomy : N/A
 ICD : N/A
 Pig Tail : N/A
 T Piece : N/A
 Chest Physio : N/A
 Treated with Inotropes : N/A
 Blood Transfusion : N/A
 Level of Consciousness : Oriented
 GCS : E4M6V5
 Vitals -TPR & BP : is stable : Yes
 Pain Score : 1/10
 On MET/PET List : No
 Risk For FALL : Yes
 Remarks : 10MODERATE RISK
 Risk For Pressure Ulcer : Yes
 Remarks : 10 HIGH RISK
 Risk for Aspiration : Yes
 Remarks : 4MODERATE RISK
 Existence of Pressure Ulcer : No
 O2 Administration : No
 SPO2(Latest reading in the shift) : 98
 DIET : Oral
 Diet-Specify the type : Diabetic
 On tube feeding : No
 IV Fluids : No
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : No
 Self Void : Yes
 Foley's : No
 Drain : No
 Bowel Opened : No
 Fluid Balance : 1245
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Next Capillary Blood glucose-checktime : 18:00
 Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : No
 Ambulation : Bed Bound
 Investigations Sent : No
 Reports Collected : Yes
 Care Plan : Yes
 Inotropes : No
 Ventilator : No



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Name:

Doctor Name:

Dialysis : No
Special Needs : Yes
Remarks : MONITOR THE VITALS
BACK CARE GIVEN
Patient & Family Education : Yes
Remarks : ATTENDERS COUNSELING DONE
Transfer : No
Medications: Yes
Remarks : AS PER DOCTORS ORDER GIVEN
Planned Procedure : No
Approved follow-up : No
Transfusion : No
Patient belongings with health care providers : No
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Charged
Remarks : STRIPS
Discharge Plan : No
Comments : BOWEL NOT OPENED D1
Hand over by : neeraja u
Handed over to : Miss Sanjukta Bose
Team Lead Hand over By : neeraja u
Team Lead Hand over To : S Sree Ram

Progress Note :
C/S/B ICU TEAM

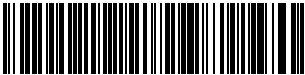
INFECTIVE EXACERBATION OF RAD
RECENT ADMISSION ON 14/3/2025
PREDIABETIC
UPPER GI BLEED - HEMOCLIP

PATIENT IS CONSCIOUS AND ORINETED
BP: 125/70
PR: 80
SPO2: 94 ON NP 3 LTRS

WBC:25K

PLAN
URINE ROUTINE
BLOOD & URINE C/S TO SEND
WATCH FOR DESATURATION / RESPIRATORY DISTRESS
CST

Treatment Note :
SEEN DURING EVENING ROUNDS



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NO FEVER
COUGH +
SPO2 - 95% ON 2L O2

ADV
PLAIN CT THORAX STAT
AUTO CPAP OVERNIGHT
SHIFT TO WARD
CONTINUE MONOCEF
MONITOR VITALS
REST AS PER CHART

Consultant Plan Notes :
74yrs/MALE

REACTIVE AIRWAY DISEASE WITH ACUTE EXACEBRATION
VIRAL LOWER RESPIRATORY TRACT INFECTION WITH RESPIRATORY FAILURE
UPPER GASTROINTESTINAL BLEED - S/P OGD SHOWED GE JUNCTION BLEEDING LESION NOTED - HEMOCLIP APPLIED [20/11/2025]
HYPERTENSION
CAME WITH C/O BREATHLESSNESS ON AND OFF SINCE 3 DAYS WORSENER SINCE 1 DAY H/O FEVER ASSOCIATED WITH COUGH WIH WHEEZE SINCE 2 DAYS
SPUTUM C/S - Moraxella catarrhalis [15.3.26]
RECENTLY ADMITTED WITH RAD EXACEBRATION FROM 14/3/26 - 17/3/26
CONSCIOUS, ORIENTED NO FEVER COUGH / WHEEZE + SPO2 - 96% ON 2L O2 CHEST CREPTS
TC 25140, HB 17.7, PLT 344000 PH 7.480, CO2 33, O2 104 CR 0.71, N 131, K 4.21
ADV ECHO, PLAIN CT THORAX STAT SHIFT TO WARD CONTINUE MONOCEF MONITOR VITALS REST AS PER CHART
FAMILY COUNSELLED IN DETAIL REGARDING CARE PLAN OF TREATMENT



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Visit No:
Doctor Name:

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 14
Remarks : high risk.
Age : 70-79 Years - 2
Elimination,Bowel and Urine : Incontinence- 2

Medications:

ON 2 OR MORE HIGH FALL RISK DRUGS - 5
Patient care Equipment : One Present- 1
Mobility : Requires assistance or supervision for mobility,transfer or ambulation - 2,Unsteady Gait- 2
Type of Assessment : Shift Assessment
Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Type: Not Applicable
Duration: Not Applicable

Skin Examination
Pallor : No
Cyanosis : No
Skin turgor : Normal turgor
Jaundice : No
Erythema : No
Petechiae : No
Oedema : No
Rash : No
Skin Lesions : No



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Surgical scars : No
Wounds : No
Burns : No
Other : REDNESS ON BOTH HANDS
Hair : Normal

Nails : Normal

Patient Acuity Tool
Stable patient 1 to 11
Moderate Risk Patient 12 to 22
Complex Patient 22 to 33
High Risk Patient >34

Total Score :21

Questionnaires

Sex : Male

A - Each Risk Factor Represents 1 Point : Sepsis (< 1 month), Serious lung disease incl. pneumonia (< 1 month), Medical patient currently at bed rest, Other risk factor

Total Tickmarks : 4

Total Score A : 4

C - Each Risk Factor Represents 3 Point : Age over 75 years

Total Tickmarks : 1

Total Score C : 3

Total Score (A+B+C+D+E) = : 7

Risk Assessment : : Highest

5 or More Highest : highest risk.

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Score : 0

CATEGORIES

PHYSICAL CONDITIONS : Difficulty Chewing - 1, Absence of Chewing - 2, Difficulty Swallowing - 1

EATING HABILTS : On Tube feeding - 3

SEATING POSITION : Unable to position appropriately while feeding due to patients health condition - 2

TOTAL SCORE : 9

COMMENTS : high risk.

Adult Pressure Ulcer Braden Scale

SENSORY PERCEPTION : 3

3 - Slightly Limited : Y

MOISTURE : 3

3 - Occasionally Moist : Y

ACTIVITY : 3

3 - Walks Occasionally : Y

MOBILITY : 3



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

3 - Slightly Limited : Y
NUTRITION : 3
3 - Adequate : Y
FRICTION AND SHEAR : 2
2 - Potential Problem : Y
TOTAL SCORE : 17- mild risk.

Forms

Nurses Endorsment Checklist
Shift : Evening to Night
Situation : Elective, NON MLC, Shift Handover
Comment / Chief Complaints / Diagnosis : C/O BREATHING DIFFICULTY
Background
Allergies : No
Remarks : UNKNOWN.
Diet : Yes
Remarks : normal
Previous Medications : No
Remarks : nil
Old reports added to TRAK : No
Remarks : nil
Consents : No
Remarks : nil
Cross Consultation : No
Remarks : nil
Procedures performed before transfer : Yes
Remarks : IV CANNULATION,
Vulnerable : Yes
Remarks : age74
Past Medical / Surgical History : RAD
DM
Intubation : NA
Ventilation Status : ON OXYGEN
ET Size and Lip Level : NA
Tracheostomy : NA
ICD : NA
Pig Tail : NA
T Piece : NA
Chest Physio : NA
Treated with Inotropes : NA
Blood Transfusion : NA
Level of Consciousness : Oriented
GCS : 15/15
Vitals -TPR & BP : is stable : Yes
Pain Score : 2
On MET/PET List : No
Risk For FALL : Yes
Remarks : 14- HIGH RISK.



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Risk For Pressure Ulcer : Yes
Remarks : 17- MILD RISK.
Risk for Aspiration : Yes
Remarks : 9- HIGH RISK.
Existence of Pressure Ulcer : No
O2 Administration : Yes
If Yes mention Litres/min : 2
SPO2(Latest reading in the shift) : 95
DIET : Oral
Diet-Specify the type : Normal
On tube feeding : No
IV Fluids : No
Blood/Blood products : No
Total Parenteral Nutrition : No
Pre Antibiotic Culture : No
Invasive Lines : Yes
Remarks : 18G VENFLON IN LEFT HAND.
Self Void : Yes
Foley's : No
Drain : No
Bowel Opened : No
Is the patient posted for any Surgery / Procedure : No
b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
Dressing Present : No
High Risk medications : No
Is Patient on Physiotherapy : No
Ambulation : Chair Bound
Investigations Sent : No
Reports Collected : No
Care Plan : Yes
Remarks : RESPIRATIONM, FALL.
Inotropes : No
Ventilator : No
Dialysis : No
Special Needs : Yes
Patient & Family Education : No
Transfer : Yes
Transfer Notes : PLAN WARD SHIFT OUT.
Medications: Yes
Planned Procedure : No
Approved follow-up : No
Transfusion : No
Patient belongings with health care providers : No
Pending Reports : No
Critical Value : No
Pharmacy / Consumables : Charged
Remarks : CONSUMABLES.



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Discharge Plan : No

Comments : PATIENT IS ON OXYGEN.

Hand over by : S Sree Ram

Handed over to : SK Anisuddin

Team Lead Hand over By : S Sree Ram

Team Lead Hand over To : Abu Haneefa B A

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : to restore respiratory system.Nursing Diagnosis : Ineffective airway clearance

Nursing Interventions : - provide head elevation up to 45 degrees high.

- monitor respiration, spo2, breathing pattern.

- oxygen administration as per need.

- encourage patient to do breathing exercise.

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : No

Nursing Care Plan discussed with Family : No

Patient Measurable Goal for the day : TO PREVENT FROM FALL. Nursing Diagnosis : Risk for fall

Nursing Interventions : - PROVIDE SIDE RAILS UP.

- KEEP BED HEIGHT LOW.

- PROVIDE PILLOW SUPPORT TO AVOID EXCESSIVE MOVEMENTS.

- PROVIDE CALL BELL FOR ANY HELP.

Diet Notes :

THE PATIENT IS ON DM LOW SALT DIET

ENERGY-1600KCAL

PROTEIN-65G

Initial Nutritional Assessment

Status : Authorised

ANTHROPOMETRIC MEASUREMENTS

Height (cms) : NOT RECORDED

Weight (kgs) : NOT RECORDED

IBW (kgs) : NaN

BMI : NaN

Recent weight loss/gain : None

GI SYMPTOMS (Recent)

Food allergies/Intolerance : No

Subjective Global Assessment rating

Nutritional intervention proposed/Diet advised : Diabetic diet with 3 meal and 3 snack as advised



Hospital No: 57233772
Name:

Visit No:
Doctor Name:

Others : Clinical notes will be updated on change of diet
Select Type : Assessment
Evaluation Criteria
Adults : New onset diabetes mellitus, Cardiac Disease **Adult Others:** lrti- sepsis rad infective exacerbation

Forms

HOURLY NURSING ROUND
Score : 0
HOURLY NURSING ROUND - DAY
CHECKPOINTS
HOURLY NURSING ROUND - NIGHT
CHECKPOINTS
Check for Sleep : 01 AM, 03 AM
Comments : Received the patient at 10:30pm
Ask for pain if any : 11 PM, 01 AM, 03 AM, 05 AM, 07 AM
Ask for Comfort or Position : 11 PM, 03 AM, 05 AM
Ask for bathroom needs : 11 PM, 01 AM, 05 AM, 07 AM
Peripery-move the phone, Call Light, Trashcan, Watercan, Over-Bed Table : 11 PM
Pump-Syringe pump, Infusion Line, IV Line : 11 PM, 03 AM, 07 AM
Nurses Endorsment Checklist
Status : Entered
Shift : Night to Morning
Situation : Elective, Shift Handover
Comment / Chief Complaints / Diagnosis : C/O BREATHING DIFFICULTY
Background
Allergies : No
Remarks : UNKNOWN.
Diet : Yes
Remarks : DM diet
Previous Medications : No
Remarks : nil
Old reports added to TRAK : No
Remarks : nil
Consents : No
Remarks : nil
Cross Consultation : No
Remarks : nil
Procedures performed before transfer : Yes
Remarks : IV CANNULATION,
Vulnerable : Yes
Remarks : age 74
Past Medical / Surgical History : RAD
DM
Level of Consciousness : Oriented
GCS : 15
Vitals -TPR & BP : is stable : Yes
Pain Score : 1



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

On MET/PET List : No
 Risk For FALL : Yes
 Remarks : 9 Moderate risk
 Risk For Pressure Ulcer : No
 Risk for Aspiration : No
 Existence of Pressure Ulcer : No
 O2 Administration : No
 SPO2(Latest reading in the shift) : 94
 DIET : Oral
 Diet-Specify the type : Diabetic
 On tube feeding : No
 IV Fluids : No
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : Yes
 Remarks : 18g cannula D-2
 Self Void : Yes
 Foley's : No
 Drain : No
 Bowel Opened : No
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist
 Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : No
 Ambulation : Ambulant
 Investigations Sent : No
 Reports Collected : No
 Care Plan : Yes
 Remarks : fall/pain/dvt
 Special Needs : No
 Patient & Family Education : Yes
 Remarks : Hand hygiene
 Transfer : No
Medications: Yes
 Remarks : As per chart
 Planned Procedure : No
 Approved follow-up : No
 Transfusion : No
 Patient belongings with health care providers : No
 Pending Reports : No
 Critical Value : No
 Pharmacy / Consumables : Charged
 Remarks : Consumables
 Discharge Plan : No
 Comments : Monitor spo2



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Hand over by : Balapdianghun
Handed over to : Rakhi M.T
Team Lead Hand over By : Balapdianghun
Team Lead Hand over To : Blessy Mol Babu
Status : Entered

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 3
Remarks : LOW RISK
Age : 70-79 Years - 2
Patient care Equipment : One Present- 1
Type of Assessment : Shift Assessment
Pain Assessments
Score: 1
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 1

Additional Pain Assessment Details

Type: Dull
Duration: Episodic
Continuous: Constant
Time: With position changes
Onset: Sudden

Associated Symptoms: None

Skin Examination
Pallor : No
Cyanosis : No
Skin turgor : Normal turgor
Jaundice : No
Erythema : No
Petechiae : No
Purpura : No



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : No
Burns : No
Other : REDNESS ON BOTH HANDS
Hair : Normal

Nails : Normal

Questionnaires

Date : 25/03/2026

Patient's Identification No. : MH009742499

Sex : Male

A - Each Risk Factor Represents 1 Point : Other risk factor

Total Tickmarks : 1

Total Score A : 1

B - Each Risk Factor Represents 2 Point : Age 60 - 74 years

Total Tickmarks : 1

Total Score B : 2

Total Score (A+B+C+D+E) = : 3

Risk Assessment : : High

3 - 4 High : 3 HIGH RISK

Adult Pressure Ulcer Braden Scale

SENSORY PERCEPTION : 4

4 - No Impairment : Y

MOISTURE : 4

4 - Rarely Moist : Y

ACTIVITY : 4

4 - Walks Frequently : Y

MOBILITY : 4

4 - No Limitations : Y

NUTRITION : 2

2 - Probably Inadequate : Y

FRICTION AND SHEAR : 3

3 - Apparent Problem : Y

TOTAL SCORE : 21 NO RISK

Forms

Nurses Endorsment Checklist

Shift : Morning to Evening

Situation : Elective,Shift Handover

Comment / Chief Complaints / Diagnosis : C/O BREATHING DIFFICULTY

Background

Allergies : No

Remarks : UNKNOWN.



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Diet : Yes
 Remarks : DM diet
 Previous Medications : No
 Remarks : nil
 Old reports added to TRAK : No
 Remarks : nil
 Consents : No
 Remarks : nil
 Cross Consultation : No
 Remarks : nil
 Procedures performed before transfer : Yes
 Remarks : IV CANNULATION,
 Vulnerable : Yes
 Remarks : age74
 Past Medical / Surgical History : RAD
 DM
 Level of Consciousness : Oriented
 GCS : 15\15
 Vitals -TPR & BP : is stable : Yes
 Pain Score : 1
 On MET/PET List : No
 Risk For FALL : Yes
 Remarks : 1 low risk
 Risk For Pressure Ulcer : No
 Risk for Aspiration : Yes
 Remarks : 0 low risk
 Existence of Pressure Ulcer : No
 Remarks : skin is intact
 O2 Administration : No
 Remarks : rom air
 SPO2(Latest reading in the shift) : 98
 DIET : Oral
 Diet-Specify the type : Normal
 On tube feeding : No
 IV Fluids : No
 Blood/Blood products : No
 Total Parenteral Nutrition : No
 Pre Antibiotic Culture : No
 Invasive Lines : Yes
 Remarks : 20 g
 Self Void : Yes
 Foley's : No
 Drain : No
 Bowel Opened : No
 Fluid Balance : normal
 Is the patient posted for any Surgery / Procedure : No
 b)If Yes, is the financial clearance obtained (As applicable) : a)If Yes, Refer the pre-operative checklist



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Dressing Present : No
 High Risk medications : No
 Is Patient on Physiotherapy : No
 Ambulation : Ambulant
 Investigations Sent : No
 Reports Collected : No
 Care Plan : Yes
 Remarks : fall ,dvt , pain
 Special Needs : No
 Patient & Family Education : Yes
 Remarks : medications
 Transfer : No
Medications: Yes
 Remarks : as per charts
 Planned Procedure : No
 Approved follow-up : No
 Transfusion : No
 Patient belongings with health care providers : No
 Pending Reports : No
 Critical Value : No
 Pharmacy / Consumables : Charged
 Remarks : consumables
 Discharge Plan : No
 Comments : cst
 Hand over by : Rakhi M.T
 Handed over to : Miss Meghana G Daddikoppa
 Team Lead Hand over By : Blessy Mol Babu
 Team Lead Hand over To : Priyanka Patari
HOURLY NURSING ROUND
 Score : 0
HOURLY NURSING ROUND - DAY
CHECKPOINTS
 Introduce yourself : 08 AM,02 PM
 Ask for pain if any : 12 NOON,01 PM,02 PM,03 PM,04 PM
 Ask for Comfort or Position : 08 AM,10 AM,12 NOON,02 PM,04 PM
 Ask for bathroom needs : 09 AM,11 AM,01 PM,02 PM,03 PM,04 PM
 Peripery-move the phone, Call Light, Trashcan, Watercan, Over-Bed Table : 08 AM,11 AM,12 NOON,02 PM,04 PM
 Pump-Syringe pump, Infusion Line, IV Line : 10 AM,01 PM,02 PM,03 PM
HOURLY NURSING ROUND - NIGHT
CHECKPOINTS
 Add Nursing Care Plan
 Status : Authorised
 Nursing Care Plan discussed with Patient : Yes
 Nursing Care Plan discussed with Family : Yes
 Patient Measurable Goal for the day : REDUCE PAIN Nursing Diagnosis : Acute pain ./ or post-operative pain
 Education and comments: : EDUCATED



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Nursing Interventions : monitor characteristics of pain, frequency, precipitating factors and relief factors

b)eliminate factors that precipitate pain

c) other analgesics 6th/8th hourly if necessary in accordance with physician orders

d)provide restful and quit environment.

d)provide restful and quit environment

Evaluation : Continue

Add Nursing Care Plan

Status : Authorised

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : PREVENT DVT RISK Nursing Diagnosis : Risk for DVT

Education and comments: : EDUCATED

Nursing Interventions : Educate in proper use and function of assistive devices.

b) Provide progressive mobilization.

c) Motivate the patient to gradually increase mobility.

Evaluation : Continue

Add Nursing Care Plan

Status : Authorised

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : PREVENT FALL RISK Nursing Diagnosis : Risk for fall

Education and comments: : EDUCATED

Nursing Interventions : Orient to surroundings on admission.

b)Provide education on how to use call bell and observe the patient demonstration.

c)Provide appropriate safety measures like sitter, display vulnerable patient alert.

d) Lower the height of bed, keep the patient near nurses station

e)Provide education on safety measures to patient and attender

Evaluation : Continue

Patient/Family Health Education

Status : Authorised

Date: : 25/03/2026

Time: : 08:52

Assessment Type : Reassessment

Educational Needs : Safe and Effective Use of Medication, side effects and Interactions,Pain Management,Activity/
Exercise,Health Maintenance,Nutrition,Risk,Falling precaution

Given Education : Safe and Effective Use of Medication, side effects and Interactions

Health Maintenance

Pain Management

Teaching Method : Lecture

Teaching Tool : Verbal Instructions

Evaluation : Applies knowledge/skills

Child Attends School : Not Applicable

Does the child has any problem at school : Not Applicable

Literacy Skills : Yes



Hospital No: 57233772
Name:

Visit No:
Doctor Name:

Progress Note :
SEEN DURING EVENING ROUNDS
NO FEVER
COUGH BETTER

TC 16700, HB 16.5 , PLT 327000

ADV
TC TOMORROW
CHEST PHYSIO
AUTO CPAP OVERNIGHT
MONITOR VITALS
REST AS PER CHART

Consultant Plan Notes :
PATIENT REVIEWED
DOING WELL
NO FEVER
COUGH +
COMPLIANT TO CPAP
SPO2 - 95% RA
CHEST RHONCHI

ECHO:
EF - 66%
PASP - 28
Normal Chamber Dimensions / Sclerotic Aortic Valve
No Regional Wall Motion Abnormality
Normal LV Systolic Function / Grade I LV Diastolic Dysfunction Present
Normal RV Function
Trivial Mitral & Tricuspid Regurgitation
No PAH / Clot / Pericardial Effusion

CT THORAX S/O Patchy subsegmental consolidation seen in the posterior / lateral basal segments of bilateral lower lobes and right apical basal segment. Centriacinar emphysematous changes seen in bilateral lung fields with an upper lobar predominance.
Well defined hypodense lesion seen in the superior pole of the left kidney and the mid / inferior pole of right Kidney measuring 21x19mm and 26x22mm respectively - likely simple cysts .Bilateral perinephric fat stranding noted .

ADV
CBC STAT
TC TOMORROW
CHEST PHYSIO
AUTO CPAP OVERNIGHT
MONITOR VITALS



Hospital No: 57233772
Name:

Visit No:
Doctor Name:

REST AS PER CHART

Diet Notes :
THE PATIENT IS ON LOW SALT DIET
ENERGY-1600KCAL
PROTEIN-55G

Forms

Fall Risk Assessment Tool

Total Fall Risk Score(sum of all points per category) : 5
Remarks : LOW RISK
Age : 70-79 Years - 2
Patient care Equipment : One Present- 1
Mobility : Unsteady Gait- 2
Type of Assessment : Shift Assessment
Pain Assessments
Score: 2
Type of Assessment: Shift Assessment
Pain Assessment Scale: Numeric Pain Scale (0-10 Scale)
Numeric Pain Scale: 2

Additional Pain Assessment Details

Since: 25/03/2026
Type: Dull
Continuous: Variable
Time: Night,Day
Onset: Gradual

Associated Symptoms: Decrease activity

Skin Examination
Pallor : No
Cyanosis : No
Skin turgor : Normal turgor
Jaundice : No
Erythema : No
Petechiae : No



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Purpura : No
Oedema : No
Rash : No
Skin Lesions : No
Surgical scars : No
Wounds : No
Burns : No
Other : REDNESS ON BOTH HANDS
Hair : Normal

Nails : Normal

Questionnaires

Status : Entered

Date : 25/03/2026

Patient's Identification No. : MH009742499

Sex : Male

A - Each Risk Factor Represents 1 Point : Abnormal pulmonary function (COPD)

Total Tickmarks : 1

Total Score A : 1

B - Each Risk Factor Represents 2 Point : Age 60 - 74 years

Total Tickmarks : 1

Total Score B : 2

Total Score (A+B+C+D+E) = : 3

Risk Assessment : : High

3 - 4 High : 3-HIGH RISK

ADULT & PAEDIATRIC - ASPIRATION RISK ASSEMENT TOOL

Status : Entered

Score : 0

CATEGORIES

TOTAL SCORE : 0

COMMENTS : LOW RISK

Forms

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : TO PREVENTED FALL Nursing Diagnosis : Risk for fall

Education and comments: : EDUCATED THE PATIENT REGARDING FALL AND PREVENTION

Nursing Interventions : - ASSESS THE PATIENT CONDITION

- PROVIDE BED SIDE RAILS

- PROVIDE HEALTH EDUCATION FALL AND PREVENTION

Evaluation : Terminate



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : TO PREVENT DVTNursing Diagnosis : Risk for DVT

Education and comments: : EDUCATED THE PATIENT REGARDING DVT AND PREVENTION

Nursing Interventions : assess the patient condition

advise the patient regarding mobilization

provide health education regarding dvt

Evaluation : Terminate

Add Nursing Care Plan

Status : Entered

Nursing Care Plan discussed with Patient : Yes

Nursing Care Plan discussed with Family : Yes

Patient Measurable Goal for the day : TO REDUCE PAINNursing Diagnosis : Acute pain ./ or post-operative pain

Education and comments: : EDUCATED THE PATIENT REGARDING PAIN AND MANAGMENT

Nursing Interventions : -ASSESS THE PATIENT CONDITION

- MONITORE PAIN SCORE BY USING SCALE

-PROVIDE COMFORTABLE POSITION

-ADMINISTERE MEDICATION AS PER DRUGCHART

Evaluation : Terminate

Patient/Family Health Education

Status : Authorised

Date : 25/03/2026

Time : 15:55

Assessment Type : Other1

Educational Needs : Pain Management,Pesonal hygiene and practitioner hand wash,Infection control related issues,Care plan and treatment,Health Maintenance,Safety,Lines/drain/catheter reconciliation,Falling precaution,Hand hygiene,Medication side effects

Given Education : HEALTH EDUCATION

Lines/drain/catheter reconciliationFalling precaution

Teaching Method : One to One

Teaching Tool : Verbal Instructions

Evaluation : Verbalize basic concept with assistance

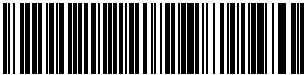
Child Attends School : Not Applicable

Does the child has any problem at school : Not Applicable

Literacy Skills : Yes

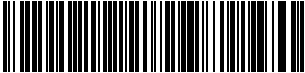
Investigation Orders

Laboratory : - Complete Blood Counts (Automated)



Hospital No: 57233772	Visit No:
Name:	Doctor Name:

MEDICATION ORDERS				
Drug / Generic Item	Dosage Qty	Frequency	Instructions/Notes	Duration
PANTOCID 40 MG/VIAL LYOPHILIZED POWDER FOR INJECTION (PANTOPRAZOLE (80007606))	1 VL	ONCE		1 DAY(S)
LEVOLIN 063 MG/25 ML RESPULES INHALATION SOLUTION (LEVOSALBUTAMOL (80006235))	1 PC	EVERY 6 HRS(6 HOURLY (Q6H))		1 DAY(S)
AUTOFUSION SET # 14289 (IV SET (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
DIGITAL THERMOMETER, MYCARE (APPLIANCE (PHARMACY NON-DRUG))	1 NUMBER	ONCE		1 DAY(S)
ANTIFLU 75 MG CAPSULE (OSELTAMIVIR (80007537))	1 CAP	EVERY 8 HRS(8 HOURLY (Q8H))		1 DAY(S)
FORAPRL 0.5 MG/2 ML RESPULE (FORMOTEROL~BUDESONIDE)	1 RESPULE	EVERY 8 HRS(8 HOURLY (Q8H))		1 DAY(S)
MONTAIR-FX (10mg, 120mg) coated tablet [10] (MONTELUKAST~FEXOFENADINE (80007046))	1 COATED TABLET	OD(0-0-1)		1 DAY(S)
NEBZMART-GFB SMARTULE SUSP INHALAT 2ML,G (GLYCOPYRROLATE (36590))	1 SUSPENSION	EVERY 8 HRS(8 HOURLY (Q8H))		1 DAY(S)
DIAPERS ADULT (XL)UNISEX B-EAZY 1'S, NUSAAC MEDIQUIPTS (SURGICALS (NON PHARMA DRUG))	2 PC	OD (0-1-0)		1 DAY(S)
UNDERPAD ABSORABLE MEDICAL(60 X90CM) ULTRACARE (UNDERPAD (PHARMACY NON-DRUG))	3 NUMBER	ONCE		1 DAY(S)
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS (SODIUM CHLORIDE (80009680))	1 BT	EVERY 8 HRS(8 HOURLY (Q8H))		1 DAY(S)
VENTRA LINE 10 CM EXT SETS#GS-3173 KAD,R (SURGICAL)	1 PC	ONCE		1 DAY(S)
SYRINGE 1 CC INSULIN 40 IU DISPOVAN, HMD (SYRINGE (PHARMACY NON-DRUG))	4 PC	ONCE		1 DAY(S)
KIPINEX FORTE 3 GM INJECTION (CEFOPERAZONE~SULBACTAM (80002516))	1 VL	ONCE		1 DAY(S)
CEFBACT 1 GM VL INJ (CEFTRIAXONE (80002546))	1 POWDERFOR INJECTION	12 HOURLY (Q12H)		1 DAY(S)
NEO-DROL 40 MG/VIAL POWDER FOR INJECTION (METHYLPREDNISOLONE (80006899))	1 VL	ONCE		2 DAY(S)
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS (SODIUM CHLORIDE (80009680))	1 BT	ONCE		1 DAY(S)
FORAPRL 0.5 MG/2 ML RESPULE (FORMOTEROL~BUDESONIDE)	1 RESPULE	ONCE		1 DAY(S)
AMLONG 5 MG TABLET 30'S (AMLODIPINE (80000465))	1 TAB	ONCE		1 DAY(S)
BED BATH TOWEL STERIX 320X320, BAPUJI (SURGICAL)	1 PC	ONCE		1 DAY(S)
MICRO POLYSYTE MONO 10CM # 13345, POLYMED (SURGICALS (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
CUROS DISIN CAP STRP NEEDLESS CONECTR,3M	2 PC	ONCE		1 DAY(S)
PANTOCID 40 MG/VIAL LYOPHILIZED POWDER FOR INJECTION (PANTOPRAZOLE (80007606))	1 VL	ONCE		1 DAY(S)
NEO-DROL 40 MG/VIAL POWDER FOR INJECTION (METHYLPREDNISOLONE (80006899))	1 VL	ONCE		1 DAY(S)
CEFBACT 1 GM VL INJ (CEFTRIAXONE (80002546))	1 POWDERFOR INJECTION	12 HOURLY (Q12H)		1 DAY(S)
SYRINGE 10 ML WITH NEEDLE DISPOVAN, HMD	4 PC	ONCE		1 DAY(S)



Hospital No: 57233772

Visit No:

Name:

Doctor Name:

(SYRINGE (PHARMACY NON-DRUG))				
AUTOFUSION SET # 14289	1 PC	ONCE		1 DAY(S)
(IV SET (PHARMACY NON-DRUG))				
POLYFLUSH PRE-FILLED SYRINGE 5 ML # 90319, POLY MEDICURE LTD	6 PC	ONCE		1 DAY(S)
(SYRINGE (PHARMACY NON-DRUG))				
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS	1 BT	12 HOURLY (Q12H)		1 DAY(S)
(SODIUM CHLORIDE (80009680))				
NEEDLE DISPOSABLE 18G X 1.1/2, DISPOVAN	4 PC	ONCE		1 DAY(S)
(NEEDLE (PHARMACY NON-DRUG))				
MONTAIR-FX (10mg, 120mg) coated tablet [10]	1 COATED TABLET	OD(0-0-1)		1 DAY(S)
(MONTELUKAST~FEXOFENADINE (80007046))				
LEVOLIN 063 MG/25 ML RESPULES INHALATION SOLUTION	1 PC	QID (1-1-1-1)		1 DAY(S)
(LEVOSALBUTAMOL (80006235))				
FORAPRL 0.5 MG/2 ML RESPULE	1 RESPULE	BD (1-0-1)		1 DAY(S)
(FORMOTEROL~BUDESONIDE)				
NEBZMART-GFB SMARTULE SUSP INHALAT 2ML,G	1 SUSPENSION	BD (1-0-1)		1 DAY(S)
(GLYCOPYRROLATE (36590))				
MUCINAC 600 MG EFFERVESCENT TABLET	1 TAB	TID. (1-1-1)		1 DAY(S)
(ACETYLCYSTEINE (80000089))				